

HOW TO OPEN A BRAIN INJURY RESIDENTIAL CARE FACILITY in Ohio

An ODH-licensed Residential Care Facility — funded through the Ohio Assisted Living Waiver.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Ohio model — read this first

In Ohio, a home that provides room, board, personal care, and limited skilled nursing to adults with brain injury is licensed by the Ohio Department of Health (ODH) as a Residential Care Facility (RCF) — Ohio's term for assisted living — under Ohio Revised Code Chapter 3721 and the rules at Ohio Administrative Code Chapter 3701-16. An RCF is a needs-based license, not an age- or diagnosis-based one, so brain-injury adults are served under the same RCF license. To be paid by Medicaid you take a second step: get your licensed RCF certified by the Ohio Department of Aging as an Assisted Living Waiver provider. This guide walks that whole path, broken down so a first-timer can follow it.

Pick the right lane — this trips people up. Three different Ohio agencies license “residential” settings, and only one is right for an adult-onset brain-injury home. ODH licenses Residential Care Facilities (assisted living) — this is your lane. DODD (Developmental Disabilities) only licenses homes for a “developmental disability,” which by law must have begun before age 22, so an adult who acquired a brain injury later generally does NOT qualify for DODD. OhioMHAS licenses mental-health/substance-use residential treatment. For a general adult brain-injury care home, the ODH RCF under ORC 3721 is the correct license.

Accreditation — strongly recommended, and how we build. Ohio does not require national accreditation to hold an RCF license, but for a brain-injury program it is a real advantage: CARF (which offers a Brain Injury specialty) and The Joint Commission signal quality to Area Agency on Aging care managers, hospital discharge planners, and families, and can strengthen your standing with payers. Every PolicyReady product is built to those standards behind the scenes. Treat accreditation as a goal from day one (see Parts 7 & 9).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It is written for someone who has never opened a facility — where we name a thing (an agency, a form, a rule, an insurance type), we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with ODH, the Ohio Department of Aging, your Area Agency on Aging, and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

Ten milestones take you from idea to a licensed, certified, paid home. Each part below is one rung.

1	Decide this is your business	A licensed, 24-hour brain-injury RCF — durable, census-based revenue (Part 1)
2	Form the business & insure	Entity, EIN, bank, and the right insurance — broken all the way down (Part 3)
3	Secure & prepare the home	Zoning, and the OAC 3701-16 physical-plant specs (Parts 3–4)
4	Set up your administrator	The RCF administrator qualifications + 9 hrs/yr CE (Part 5)

5	Build & train staff	Personal-care & medication training, first aid, background checks (Part 5)
6	Write your policies	P&P Manual + the OAC 3701-16 operating standards (Parts 4–6)
7	Get the ODH license	Apply ≥60 days out; fire & pre-licensure inspections; license (Part 6)
8	Certify & enroll for the waiver	ODA Assisted Living certification + Ohio Medicaid (PNM) enrollment (Part 8)
9	Get your first residents	Area Agencies on Aging + OSU Dodd / hospital discharge planners (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, all in one place (Part 11)

Rule of thumb. Start your administrator and your State Fire Marshal / plan-review conversation early — the building and fire approvals gate your license, and the application must be in at least 60 days before you open. Do the cheap, reversible steps (form the entity, sign a contingent lease) first, and get WebCheck background checks started right away.

Before you begin — a realistic picture

Be honest with yourself about the scale of this before you spend a dollar. This is not a passive real-estate play — it is a licensed, 24-hour healthcare operation for adults who cannot fully care for themselves, and Ohio holds you accountable for their safety around the clock. Expect several months from decision to your first admitted resident: the building and fire approvals, the ODH pre-licensure survey, the Ohio Department of Aging certification, and the Medicaid enrollment each take real time and cannot be rushed by paying more. Expect to carry payroll (including nursing coverage for medications), insurance, and the mortgage or lease before a single Medicaid per-diem arrives, so you need working capital to reach a break-even census. And expect to become a documentation business as much as a care business — surveyors and Area Agency on Aging care managers judge you on what you can show in writing. None of this should scare you off; brain-injury adults are genuinely underserved in Ohio and the Assisted Living Waiver is built to fund exactly this care. But go in with your eyes open, a cash cushion, and patience for the sequence.

A one-paragraph roadmap of the Parts

Here is the whole guide in a breath. Part 1 defines the business — an ODH-licensed Residential Care Facility — and Part 2 introduces the four agencies that regulate, pay, and refer to you. Part 3 walks you through forming the entity, insuring it, and securing the home, and Part 4 covers the building itself: the OAC 3701-16 physical-plant, bathroom, temperature, and fire-code specs. Part 5 is people — your administrator, staff training, the Ohio medication rule, and background checks — and Part 6 takes you through the ODH license application, inspections, and issuance. Part 7 follows a resident from referral to discharge, and Part 8 is how you actually get paid: certifying with the Ohio Department of Aging, enrolling with Medicaid, and billing the Assisted Living Waiver per-diem. Part 9 sets out costs and timeline, Part 10 shows where your first residents come from, and Part 11 gathers every contact and every “verify” into one place. Read it once through, then work it as a checklist.

PART 1

What This Business Is

In short: An ODH-licensed Residential Care Facility for brain-injury adults — accommodation, supervision, personal care, and limited skilled nursing. It's a real-estate + 24-hour-care business.

Ohio law (ORC 3721.01) defines a Residential Care Facility as a home that provides accommodation, supervision, and personal care to unrelated adults, and that provides skilled nursing care on a limited, intermittent basis to at least one resident. It's the assisted-living license — you're licensed for the level of care your residents need, not for a particular diagnosis — so a home built for adults with brain injury operates under the same RCF license as any other.

Licensed RCF vs. Medicaid-certified waiver provider

Term	What it means
Residential Care Facility (RCF)	The ODH license itself — you must have this first, whether or not you ever bill Medicaid
Assisted Living Waiver certification	A second step: your licensed RCF is certified by the Ohio Dept. of Aging so you can bill the waiver
Personal care vs. skilled nursing	RCFs deliver personal care (help with daily living) plus skilled nursing only on a limited/intermittent basis

Serving residents who wander

Ohio's RCF rules don't create a separate “memory care” or “secure unit” license. If a resident is prone to wandering (common after brain injury), any locked-egress arrangement is handled through the Ohio Building and Fire Codes (delayed-egress / special-locking provisions) at plan review, plus your resident-rights and care-planning obligations — not through a dedicated ODH secure-unit rule. Decide early whether you'll run a secured setting, and treat the locking as a fire-code plan-review item with the State Fire Marshal.

Key terms. ODH = Ohio Dept. of Health (your licensor). RCF = Residential Care Facility (Ohio assisted living). ORC = Ohio Revised Code (the statute). OAC 3701-16 = the RCF rule chapter. ODA = Ohio Dept. of Aging (runs the waiver). ODM = Ohio Dept. of Medicaid. ALW = Assisted Living Waiver. AAA / PAA = Area Agency on Aging / PASSPORT Administrative Agency (your assessment & referral hub). NFLOC = Nursing Facility Level of Care. PNM = Provider Network Management (Medicaid enrollment). BCI/WebCheck = the background-check system. NPI = National Provider Identifier. CARF/TJC = the two accreditors.

PART 2

Who Regulates You (and What Each One Does)

In short: ODH licenses you, the State Fire Marshal clears your building, the Ohio Dept. of Aging + your Area Agency on Aging certify you and send residents, and the Long-Term Care Ombudsman advocates for residents.

ODH — Ohio Department of Health (your licensor)

The Ohio Department of Health, Bureau of Survey & Certification, issues your RCF license, writes the OAC 3701-16 standards, reviews your plans, and conducts the pre-licensure and ongoing surveys (at least every 15 months). Phone (614) 752-9524; LICCERT@odh.ohio.gov; odh.ohio.gov.

State Fire Marshal / local fire department (fire & life safety)

Before ODH licenses you, the State Fire Marshal or your local fire department inspects the building against the Ohio Building and Fire Codes for your occupancy. For a brain-injury home whose residents can't all self-evacuate, this matters even more — engage them early, especially if you'll lock any exits.

Ohio Dept. of Aging + your Area Agency on Aging (who pay you / send residents)

The Ohio Department of Aging (ODA), with the Ohio Department of Medicaid, runs the Assisted Living Waiver and sets the rates. Locally, your Area Agency on Aging (also called a PASSPORT Administrative Agency) does the assessments, certifies you (with an on-site review), enrolls waiver residents, and manages their care — which makes the AAA both your certifier and your main referral source. Find-your-AAA line 1-866-243-5678.

Long-Term Care Ombudsman

The Ohio State Long-Term Care Ombudsman is a free, independent advocate for residents of assisted living, nursing, and adult-care settings — you post the number for residents (1-800-282-1206; regional offices are hosted by the AAAs). You must report suspected abuse, neglect, or exploitation.

PART 3

Form Your Business & Secure the Home — Step by Step

In short: *This is where people get stuck. We break each piece down: what it is, where to go, and exactly what to do.*

3.1 Form your legal entity

An “entity” is the legal business (usually an LLC) that holds the license and bank account and shields your personal assets. In Ohio you form it at the Secretary of State.

- 1. Go to the Ohio Secretary of State.** File online at OhioBusinessCentral (sos.state.oh.us; Business Services 1-877-767-3453).
- 2. File Articles of Organization (Form 610).** The filing fee is \$99. Name the LLC and list a statutory agent (receives legal mail — can be you).
- 3. Adopt an Operating Agreement** — the internal document setting out ownership, management, and money. Ohio doesn't file it, but your bank and partners will want it.
- 4. Register a trade name if you'll use one** — file Form 534A with the SOS if your public name differs from the LLC's legal name.

Two nice Ohio simplifications. Ohio does NOT require an annual report for LLCs — there's no yearly state filing to keep the LLC active. And Ohio does not require newspaper publication to form an LLC. Forming the entity is essentially a one-time \$99 online filing.

3.2 Get your EIN (federal tax ID)

An EIN is your business's federal tax number — free from the IRS, and you need it for the bank, payroll, and Medicaid enrollment.

- 1. Go to IRS.gov** and search “Apply for an EIN online” (the EIN Assistant). It's free — never pay a third-party site.
- 2. Apply online** with the responsible party's name and SSN/ITIN and your entity info.
- 3. Save the confirmation (CP-575)** — you get the EIN immediately.

3.3 Open a business bank account

What to have ready:

- EIN confirmation from the IRS
- Your Articles of Organization and the entity's charter number
- Operating Agreement (LLC)
- A Beneficial Ownership form (owners of 25%+ and a control person)
- Government photo ID, SSN, DOB, home address for each owner/signer
- An opening deposit (often ~\$100)

Where to go: a major bank with lots of Ohio branches — Chase, Huntington (a large Ohio-based bank), Fifth Third, KeyBank, or PNC, or a local credit union. Compare monthly fees and how to waive them, transaction/cash-deposit limits, branch access, online bill pay, and whether they offer a business line of credit.

3.4 Get your insurance — what you need, who sells it, and what to ask

An assisted living home cares for vulnerable adults in a building, so you need several kinds of coverage — not one policy:

Coverage	What it protects against	Must-have?
General Liability (GL)	A visitor/family slip-and-fall or property damage on your premises	Yes — foundational
Professional Liability (malpractice / E&O)	Claims about the CARE — supervision, med errors, neglect	Yes — top healthcare coverage
Abuse & Molestation	Allegations of abuse by staff or others (GL)	Yes — high exposure

	excludes this!)	
Commercial Property	The building, contents, equipment; add Business Interruption	Yes if you own; often if you lease
Commercial Auto (incl. hired/non-owned)	Transporting residents; staff driving own cars for work	Yes if you drive residents
Workers' Compensation	Staff injuries (lifting/transferring is high-risk)	Required in Ohio — through the state fund (BWC)
Cyber Liability	A breach of protected health information (HIPAA)	Strongly recommended

Ohio workers' comp is different — it's a state fund. Ohio is a “monopolistic” workers'-comp state: you buy workers' compensation from the Ohio Bureau of Workers' Compensation (BWC), not from a private insurer. Set up your BWC policy as soon as you have employees (bwc.ohio.gov).

Real places to get it

Buy through an independent agent/broker who accesses specialty carriers. Carriers that write senior-care/residential-care risk include Philadelphia Insurance (PHLY), CNA, Markel, Berkshire Hathaway GUARD, Crum & Forster, Selective, and Nationwide (an Ohio-based insurer). Specialty programs/brokers for this niche include Caitlin Morgan, Gallagher, AssuredPartners, Professional Insurance Group, and XInsurance. To find a local Ohio agent, use the “Trusted Choice” Find-an-Agent directory at trustedchoice.com (filter healthcare/senior-care), or ask a licensed peer. Verify each carrier writes assisted living in Ohio before relying on a quote.

What to ask the insurance agent

“Do you write residential care facilities / brain-injury homes — which carriers?”

“Is professional liability included — claims-made or occurrence — and what limits?”

“Is abuse & molestation included, at what limit, sub-limit or full?”

“Are GL and professional liability shared limits or separate?”

“What limits does the waiver contract or my landlord/lender require?”

“How do I set up my Ohio BWC workers'-comp account, and what will it cost?”

3.5 Secure & zone the home

- ☐ Call your city/county ZONING office: is a residential care use for [n] residents allowed at this address, or do you need a use permit? Ohio group homes for people with disabilities generally have residential-use protection under fair-housing law — ask them to confirm in writing.
- ☐ Confirm the building can meet the OAC 3701-16 physical-plant specs (Part 4) and be made accessible.
- ☐ Confirm you can get a certificate of occupancy and a passing State Fire Marshal inspection for your resident count (and any locked-egress plan).
- ☐ If leasing, make sure the lease allows your use and resident count, and make it contingent on licensure.

3.6 Map ownership + background checks

Identify every owner and controlling person — you disclose them on the ODH application, and every employee must clear a BCI (and where required FBI) WebCheck background check under ORC 3721.121 (Part 5.4).

3.7 Get your NPI (National Provider Identifier)

An NPI is a free national ID payers use to identify you. Your FACILITY needs a Type 2 (Organizational) NPI:

1. **Create an I&A account** at nppes.cms.hhs.gov (Identity & Access) for the person who'll manage the number (usually an owner).
2. **Log into NPPES** and start a new application; choose Type 2 (Organizational).
3. **Enter your legal name, EIN, and address** and pick the taxonomy that fits an assisted living / residential facility.

- 4. Submit — it's free** and usually processes in about 10 business days. Save your NPI; you'll need it for Medicaid enrollment (Part 8).

3.8 Put the pieces in the right order

The steps in this Part have a natural sequence, and doing them out of order is where first-timers waste weeks. You need the entity (3.1) before you can get the EIN (3.2); the EIN before the bank account (3.3) and before payroll and Medicaid enrollment; and the entity and EIN before your NPI (3.7). Insurance (3.4) can be quoted early but is bound once you have an address and a resident count. Zoning and the building (3.5) should start in parallel from day one because they gate everything downstream. Here is a sensible order:

- 1. Form the LLC and adopt the operating agreement** (3.1) — the \$99 online filing that creates the business.
- 2. Get the EIN the same week** (3.2) — it's instant and unlocks the bank and payroll.
- 3. Open the business bank account** (3.3) — keep every business dollar out of your personal accounts from the first day.
- 4. Start zoning and the fire-code conversation** (3.5) — in parallel, before you sign anything you can't undo.
- 5. Quote insurance and set up your Ohio BWC account** (3.4) — bind once the address and bed count are set.
- 6. Apply for the Type 2 NPI** (3.7) — free, and needed later for Medicaid; there's no reason to wait.

What this means for you. Keep a single dated folder — paper or cloud — with your Articles of Organization, EIN letter, operating agreement, insurance binders, BWC number, NPI, and every zoning and fire response. This is the same folder that becomes your ODH application packet (Part 6) and your ODA certification file (Part 8). Building it as you go, rather than reconstructing it under a deadline, is the single biggest time-saver in this whole process.

Common early mistakes to avoid. Don't sign a non-contingent lease before zoning and the fire path are confirmed. Don't pay a third-party site for an EIN or NPI — both are free from the government. Don't co-mingle personal and business money, which weakens the liability shield your LLC exists to provide. And don't skip the operating agreement just because Ohio doesn't file it — your bank, partners, and lender will ask for it.

PART 4

The Home — Physical Plant, Safety & Fire Code (OAC 3701-16)

In short: *Ohio gives you real numbers for bedrooms, bathrooms, and temperature, and runs fire safety through the Ohio Building & Fire Codes. Here's what each requirement means and how it's checked.*

“Physical plant” just means the building itself and everything fixed in it — the rooms, plumbing, kitchen, and fire systems. ODH inspects the health/physical side and the State Fire Marshal (or local fire department) inspects fire & life safety before you're licensed. The rules live in OAC Chapter 3701-16 (note: 3701-16 is the RCF chapter — 3701-17 is nursing homes). Below, each requirement is followed by what it means for you.

4.1 Bedrooms & space (OAC 3701-16-14)

- ☐ Single-occupancy bedroom: at least 100 sq ft (facilities built 1964 or later; 80 sq ft in older buildings)
- ☐ Multiple-occupancy bedroom: at least 80 sq ft PER resident (70 sq ft in older buildings)
- ☐ No more than four residents per bedroom
- ☐ Each resident has their own bed and personal storage; a resident call system
- ☐ Living/common and dining areas sized for the residents

What this means for you: measure every bedroom before you sign a lease. In a modern building a single room needs a full 100 sq ft (about 10 by 10), and a shared room needs 80 sq ft per person. Brain-injury residents often use wheelchairs or walkers, so leave real turning room beyond the minimum.

4.2 Bathrooms & temperature (OAC 3701-16-14 / -16)

- **Bathrooms:** at least one toilet, one shower or bathtub, and one sink for every 8 residents.
- **Safety:** grab bars at toilets and in tubs/showers, non-slip surfaces, and features for residents using wheelchairs/walkers/transfer benches.
- **Temperature:** keep resident areas between 71°F and 81°F, with a written plan for out-of-range response and HVAC repair arranged within 48 hours.

What this means for you: think like a surveyor walking your bathrooms and kitchen. Bathroom ratios, safe water, working heat/cooling, and safe storage of chemicals and medications are the common early findings. Verify your water temperature, add grab bars, and lock up chemicals and medications before you apply.

4.3 Fire & building safety (OAC 3701-16-13)

- ☐ An automatic fire-extinguishing (sprinkler) system and a fire-alarm system conforming to the Ohio Building Code
- ☐ Carbon-monoxide detectors per the Ohio Fire Code (section 915)
- ☐ Fire drills, with alarm transmission to the fire department or a monitoring station
- ☐ A certificate of occupancy and a passing State Fire Marshal (or local fire department) inspection before licensure
- ☐ A written disaster-preparedness plan and staff trained to evacuate non-ambulatory residents; keep every inspection report in one dated compliance binder

Confirm your occupancy classification early. *Ohio regulates RCF construction through the Ohio Building and Fire Codes rather than adopting the NFPA 101 Life Safety Code by name. Your occupancy group (assisted-living facilities are often Institutional Group I-1) drives the exact sprinkler and alarm requirements — confirm it with your plan reviewer and the State Fire Marshal for your specific building and bed count before you renovate.*

4.4 Brain-injury environmental design (why it matters here)

Ohio's RCF rules don't spell out brain-injury-specific design, so build it in yourself: clear, low-clutter walking paths to reduce falls; consistent, labeled locations for personal items (memory and orientation support); door alarms or a

code-compliant secured setting if any resident wanders; reduced noise and visual overstimulation in common areas; and a quiet space a dysregulated resident can use. This is both good care and part of keeping residents safe.

4.5 Walk the building like a surveyor — a pre-inspection checklist

Before you request the pre-licensure survey, walk every room with this list in hand as if you were the ODH surveyor and the fire inspector. Fixing these yourself first is far cheaper than a return visit and a delayed opening.

Bedrooms & living space:

- ☐ Every single room measures at least 100 sq ft (80 in pre-1964 buildings); shared rooms at least 80 sq ft per resident
- ☐ No bedroom holds more than four residents; each has a bed, personal storage, and a working call signal
- ☐ Common and dining areas are sized for your licensed count, with real wheelchair turning room

Bathrooms, water & climate:

- ☐ At least one toilet, one tub/shower, and one sink for every eight residents
- ☐ Grab bars at every toilet and in every tub/shower; non-slip surfaces throughout
- ☐ Hot-water temperature at fixtures is safe (not scalding); resident areas hold between 71°F and 81°F

Fire & life safety:

- ☐ Sprinkler and fire-alarm systems tested and certified; carbon-monoxide detectors per Ohio Fire Code 915
- ☐ Certificate of occupancy in hand; State Fire Marshal (or local fire dept.) inspection passed
- ☐ Written disaster plan posted; staff trained to evacuate non-ambulatory residents; any locked egress cleared at plan review

Safe storage & documentation:

- ☐ All medications and hazardous chemicals locked and separated from resident-accessible areas
- ☐ Every inspection report, test certificate, and heating-system check filed in one dated compliance binder

What this means for you. *The most common early survey findings are mundane and preventable: an unlabeled or unlocked chemical closet, a missing grab bar, water that runs too hot, or a call system that doesn't reach a bathroom. Catching them on your own walk-through turns a citation into a five-minute fix and keeps your opening date intact.*

PART 5

Staff, the Administrator & Required Training — by Position

In short: *Ohio's rules are flexible on who can be administrator but specific on training. Here they are, one by one — and the medication rule is a real Ohio quirk you must plan around.*

5.1 Your RCF administrator (OAC 3701-16-06)

Every RCF must have an administrator (who may be you). Ohio does NOT require a separate RCF administrator license — you qualify one of several ways:

- 1. Meet the floor.** At least 21 years old.
- 2. Meet one qualification pathway.** A licensed Ohio nursing home administrator (through the BELTSS board); OR 3,000 hours of direct operational responsibility in senior housing/health-care facilities; OR 100 post-high-school credit hours in gerontology or health care; OR a bachelor's degree; OR status as a licensed health professional.
- 3. Be present enough.** The administrator must be in the facility at least 20 hours a week during 8 a.m.–6 p.m., and accessible at all other times; designate a qualified acting administrator for absences.
- 4. Keep up your continuing education.** At least 9 hours a year in gerontology, health care, business administration, or RCF operation.

You may already qualify. Because a bachelor's degree (in any field), a health-professional license, or 3,000 hours of senior-care operational experience each satisfy the requirement, many first-time owners can serve as their own administrator without a special license. Confirm your pathway against OAC 3701-16-06.

5.2 Medication — Ohio's default-self-administration rule (OAC 3701-16-09)

This is the Ohio quirk to plan around. In an RCF the default is that residents self-administer their own medications, and staff are barred from administering medications except as the statute (ORC 3721.011) allows. When medication IS administered, it must be by a registered nurse; a licensed practical nurse who holds a Board of Nursing-approved medication-administration course; a physician; or another person authorized by law. Unlicensed staff may assist with self-administration after training. Because brain-injury residents often cannot safely self-administer, plan your nursing coverage (an RN or a properly-trained LPN) around actual medication administration from day one — this shapes your staffing and budget.

5.3 Direct-care staff & coverage (OAC 3701-16-05 / -06)

- **Presence:** at least one qualified personal-care staff member on duty at all times, and always enough staff present to evacuate residents.
- **Overnight:** Ohio does not require awake overnight staff by rule — the night staff member may be on-call if you have a resident call system — but a brain-injury home's acuity often makes awake overnight coverage necessary; document a staffing plan a surveyor can see meets your residents' needs.
- **First aid:** staff must complete first-aid training within 60 days of hire, and at least one first-aid-trained personal-care staff member must be present at all times.
- **Initial training:** personal-care staff must be trained in correct techniques, observation, and communication before working unsupervised.
- **Annual & specialty training:** 8 hours of continuing education a year; plus dementia/cognitive-impairment training (2 hours within 14 days of assignment + 4 hours annually) and serious-mental-illness training (2 hours + 4 hours annually).

5.4 Brain-injury competence & background checks

Ohio requires dementia and mental-illness training but no brain-injury-specific module — so make brain-injury competencies part of your own program: cognitive stimulation, wander- and personal-safety, responding to sudden

or dysregulated behavior, and communication strategies for cognitive/speech impairment. On screening, run these before hire:

- 1. Request a BCI check via WebCheck.** The administrator must obtain an Ohio Bureau of Criminal Investigation criminal-records check (fingerprint-based, through a WebCheck provider) for each applicant.
- 2. Add the FBI check where required.** An FBI fingerprint check is required if the applicant has not lived in Ohio for the past 5 years (and may be requested regardless).
- 3. Mind the disqualifying offenses & the 60-day rule.** ORC 3721.121 lists disqualifying offenses; conditional employment is allowed pending results, but must end if results aren't received within 60 days.
- 4. Document everything.** Keep the dated results in each employee file; a surveyor will ask to see them. (WebCheck fees are roughly \$30 BCI + \$30 FBI plus the vendor fee — verify current.)

5.5 What belongs in every employee file

Your staff records are inspected. Build each file the same way so a surveyor can open any one and see a complete, dated picture. What this means for you: assemble the file on the hire date, not when the survey is announced.

- ☐ Application, resume, and verified references
- ☐ BCI (and FBI where required) WebCheck results, dated — with any conditional-hire and 60-day tracking
- ☐ First-aid certification (completed within 60 days of hire) and CPR if you require it
- ☐ Initial competency sign-off (personal-care techniques, observation, communication) before unsupervised work
- ☐ Dementia/cognitive-impairment training (2 hrs within 14 days + 4 hrs annually) and serious-mental-illness training (2 hrs + 4 hrs annually)
- ☐ Your brain-injury competency module and the annual 8 hours of continuing education, dated each year
- ☐ Medication-administration credential for any nurse who administers meds (RN, or LPN with the Board of Nursing course)
- ☐ Job description, TB screening / health documentation per your policy, and signed acknowledgment of your policies

5.6 A worked example — staffing a small brain-injury home

To make the staffing rule concrete, picture a six-bed home. You need at least one qualified personal-care staff member on duty at all times, always enough staff present to evacuate every resident, and a first-aid-trained staff member present at all times. Covering three shifts a day, seven days a week, means roughly 4.2 full-time-equivalent direct-care positions just for continuous single coverage — and a brain-injury census with transfers, wandering risk, and behaviors usually pushes you to two staff on the busy day and evening shifts. Layer on your nursing coverage for medication administration (an RN, or an LPN with the medication course), because most of your residents will not be able to self-administer safely, and your administrator's required 20 hours a week on-site. Budget for that real headcount from day one; understaffing is both a survey risk and a safety risk.

PART 6

Get Your ODH License — Step by Step

In short: *Submit your RCF application at least 60 days before opening, pass the plan review, the State Fire Marshal inspection, and the ODH pre-licensure survey, and receive your license.*

6.1 Before you apply

- ☐ Entity formed, EIN, NPI, insurance bound (including Ohio BWC workers' comp)
- ☐ Home secured and brought up to OAC 3701-16 specs; certificate of occupancy and fire path confirmed
- ☐ Administrator identified and qualified; nursing coverage planned for medication administration
- ☐ Staff training plan and policies & procedures written (your P&P Manual)
- ☐ WebCheck background-check system in place for staff

6.2 The application (OAC 3701-16-03)

- 1. Get the current RCF application packet.** Request it from ODH Bureau of Survey & Certification — LICCERT@odh.ohio.gov or (614) 752-9524.
- 2. Submit at least 60 days before opening.** File the application with the required attachments: a financial-solvency statement, ownership information, floor plans, your certificate of occupancy, the State Fire Marshal (or local fire department) inspection report, and a heating-system inspection.
- 3. Pay the license fee.** The statute sets it at \$320 for each 50 persons (or part thereof) of licensed capacity, for both initial licensure and annual renewal. Confirm the exact current posted amount with ODH.
- 4. Pass the pre-licensure inspections.** ODH inspects the facility and the State Fire Marshal (or local fire department) inspects fire & life safety before the license issues.
- 5. Receive your RCF license.** Then keep it current — ODH surveys at least every 15 months (up to 30 months for facilities meeting compliance criteria).

Note on change-of-operator. *If you buy an existing licensed home rather than start fresh, a change-of-operator carries a much larger fee (\$3,200) and a surety bond of \$10,000 per licensed bed. Factor that in if you're acquiring rather than building.*

6.3 Timeline

Because the application must be in at least 60 days before you open, and the plan review, fire inspection, and pre-licensure survey each take time, plan for several months end to end. Start the administrator, the fire-code/occupancy conversation, and the background checks in parallel while the application is pending.

Ask before you file. *Call ODH at (614) 752-9524 to confirm the current packet, the exact fee, and which attachments your application needs — and the State Fire Marshal about the occupancy classification and fire path for your building. A complete first submission saves weeks.*

PART 7

The Resident's Journey — Referral to Discharge

In short: A waiver client is assessed and referred by an Area Agency on Aging care manager; you confirm the fit, admit under a residency agreement, deliver and document the care plan, and plan for the least-restrictive setting. Here is the whole journey, stage by stage.

Stage 1 — Referral comes in

A referral comes from an Area Agency on Aging (AAA) care manager, or a hospital or rehab (e.g., OSU Dodd, MetroHealth) discharge planner (Part 10). Someone asks whether you have a bed for a specific person on the Assisted Living Waiver.

What you do:

- Take the basics (age, diagnosis, level of care, funding, target move date). Ask for the person's assessment and care plan. Tell them you're an ODA-certified Assisted Living provider and your current openings and whether you offer the memory-care service.

Stage 2 — Pre-admission screening (can you safely serve this person?)

Before you say yes, confirm the resident fits an RCF, your capacity, and your abilities. Their needs can't exceed what an RCF can provide (limited/intermittent skilled nursing — not a nursing facility), and if they wander you must have the code-compliant setting and staffing to keep them safe.

What to review:

- Diagnoses and behaviors (aggression, elopement/wandering, swallowing/choking risk); mobility and transfer needs; medications and who will administer them (your nurse); and whether your trained staffing can meet all of it safely. When in doubt, ask the AAA care manager and the resident's physician.

Stage 3 — The AAA sets the plan

For waiver clients, the AAA — not you — determines Nursing Facility Level of Care through the state assessment, confirms financial eligibility, and builds the person-centered care/service plan that authorizes the assisted-living service and the service package (basic or memory care). The member chooses among certified providers.

Stage 4 — Admission & the residency agreement

On move-in, the resident (or their representative) signs a residency agreement covering services, resident rights, fees, who pays room and board, and how residency can end. Give a rights orientation and open a resident record.

The resident record must include:

- Health information and diagnoses; a current medication list with allergies; advance directives / power of attorney; physician and AAA-care-manager contacts; emergency contacts and the representative; and the signed agreement and consents.

Stage 5 — Build the care plan (the roadmap)

Within your policy timeframe, develop and follow the resident's care plan — the day-to-day plan for THIS person, aligned to the AAA's service plan. For a brain-injury resident it typically covers cognitive-stimulation activities, personal- and wander-safety measures, help with the activities of daily living, medication administration by your nurse, nutrition and any special diet, and behavior supports. Coordinate with the physician and AAA, and have the resident/representative sign off.

Stage 6 — Daily life & care

Deliver the plan: structured daily routines, ADL support, cognitive and social activities, community outings as appropriate, three meals a day, and safe medication administration — all mapped to the resident's goals. Maintain your required coverage, and document progress notes so you can show the plan is being followed.

Stage 7 — Reviews, incidents & rights

Review and update the care plan as needs change and whenever the AAA reassesses (which can change the service package). Run your quality program with incident reports; protect resident rights; and report any suspected abuse, neglect, or exploitation immediately. Everything is documented and followed up.

Stage 8 — Discharge & transition

When a resident's needs change or goals are met, coordinate a safe transition with the AAA and family — to a less-restrictive setting, a higher level of care (a nursing facility if they can no longer be safely served in an RCF), or the community — following your termination-of-residency policy and giving proper written notice. Send the receiving provider the records they need, and document the transition.

The documentation trail behind each stage

Every stage above leaves a paper (or electronic) trail, and that trail is what protects the resident, satisfies the AAA and ODH, and supports your Medicaid billing. What this means for you: build a simple resident record structure so nothing is reconstructed after the fact.

- ☐ Referral & screening notes — who referred, what you reviewed, and your admit/decline decision with the reason
- ☐ Signed residency agreement, resident-rights orientation, and all consents
- ☐ The AAA service plan and your resident-specific care plan, both signed and dated
- ☐ Daily progress notes, ADL and activity records, and the medication administration record (MAR)
- ☐ Incident reports, care-plan reviews after each reassessment, and any abuse/neglect reports made
- ☐ Discharge/transition summary and the records sent to the receiving provider

What this means for you. A brain-injury resident's needs shift over time, and the AAA reassessment can move them between the basic and memory-care service packages — which changes both the care you deliver and the per-diem you bill (Part 8). Treat every reassessment as a trigger to update the care plan, the staffing, and your billing, and to re-confirm the resident can still be safely served in an RCF rather than a nursing facility.

PART 8

How You Get Paid — the Assisted Living Waiver, Step by Step

In short: License your RCF, get certified by the Ohio Dept. of Aging as an Assisted Living provider, enroll as an Ohio Medicaid provider, and bill the per-diem. Medicaid pays for the care — never room & board. Verify each resident's waiver before admission.

Summary of the sources:

Source	What it is	Key fact
Assisted Living Waiver (ALW)	Medicaid HCBS waiver (ODA + ODM)	Pays a per-diem for the service in your RCF; the main Medicaid residential route (age 21+, NFLOC)
Private pay	Family/self-pay	Always covers room & board; may cover all care for non-waiver residents
Veterans (VA)	Aid & Attendance, VA residential care	May help fund a veteran's care — verify eligibility

8.1 How a brain-injury adult is funded in Ohio

Ohio has no dedicated brain-injury Medicaid waiver, so the main residential route is the Assisted Living Waiver: age 21 or older, meets Nursing Facility Level of Care, and financially eligible (in 2026, countable assets no more than \$2,000 and monthly income within 300% of the SSI benefit rate — about \$2,982). The Ohio Home Care Waiver covers people under 60 with disabilities (including non-developmental brain injury) but only for in-home care — it does NOT pay for a residential setting's room and board. Adult-onset brain injury generally does not qualify for the DODD (developmental-disability) waivers, which require onset before age 22. So for a residential home, the Assisted Living Waiver is your route.

8.2 License, certify, get your NPI, then enroll

- 1. License the RCF first (Part 6).** You cannot be certified for the waiver until ODH has licensed the building.
- 2. Have your Type 2 NPI ready (Part 3.7).**
- 3. Enroll as an Ohio Medicaid provider.** Use the Provider Network Management (PNM) module (ohpnm.omes.maximus.com), which replaced the old MITS front end.
- 4. Get ODA Assisted Living certification.** Review the provider conditions (OAC 173-39-02.16), complete BCI/FBI checks and the HCBS Settings Evaluation, create an OH|ID account, write the required policies, and apply through PNM's Provider Certification Wizard (the 2025 application fee was \$730 plus a card fee — confirm current).
- 5. Pass the AAA pre-certification review.** Your local Area Agency on Aging visits the site to validate your policies and documentation (they have up to 60 days) and recommends approval to ODA, which then issues your provider contract.

8.3 The rate & what's covered

The Assisted Living Waiver pays a per-diem (daily) rate for the service, as “payment in full” — you may not balance-bill the resident the difference between the Medicaid rate and a private charge. Ohio moved to a simplified structure effective January 1, 2024:

Service	Per day (verify current)	What it is
Basic Assisted Living	\$130 / day	Personal care, 24-hour response, meals, housekeeping, activities
Memory Care	\$155 / day	Enhanced staffing, dementia training, therapeutic activities

Confirm the current rate with ODA before you rely on it (a further increase may have taken effect since 2024). The service package the AAA authorizes drives which rate you bill.

Room & board is NOT covered by Medicaid. The waiver pays only the service. The resident pays room and board from their own income, set by state rule at the SSI federal benefit rate minus a \$50 personal-needs allowance — for 2026 that's \$994 – \$50 = \$944/month. You collect room & board directly from the resident by a written monthly due date. Build this into your admission and your residency agreement.

How you actually bill (the claims cycle)

1. **Verify eligibility & the authorization** — confirm the member's waiver enrollment and the authorized service package before and during the stay.
2. **Submit the claim** — the per-diem through Ohio Medicaid (via PNM/the Medicaid claims system), using your NPI and the authorized service.
3. **Watch timely filing** — Ohio Medicaid fee-for-service generally uses a 365-day window from the date of service; confirm the current billing rule.
4. **Collect room & board** from the resident by your written due date, separate from the Medicaid claim.

Ask ODA / your AAA:

- Which service package this resident is authorized for; the current per-diem; how a reassessment changes the package and rate; the exact room-and-board figure; and the claims and timely-filing details.

8.4 Private pay & veterans

- **Private pay:** non-waiver residents (and every resident's room & board) pay privately — Ohio assisted living commonly runs roughly \$4,500–\$5,000/month, and a specialized brain-injury home typically prices above that; set a clear rate and a written residency/financial agreement, reviewed by counsel.
- **Veterans:** VA Aid & Attendance (an add-on to the VA pension) can help fund a veteran's care — 2025–2026 maximums are about \$2,424/month for a single veteran and about \$1,558/month for a surviving spouse (verify on VA.gov). The VA Polytrauma/TBI System of Care is also a resource for veterans with brain injury.

PART 9

What It Costs & How Long It Takes

In short: Budget the home, the administrator and staff training, nursing coverage for medications, background checks, and the ODH fees — and consider accreditation. Plan a few months through licensing, then certification.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
The home	Lease deposit + first month (or purchase); accessibility mods; furnishings; life-safety systems	Varies by market/size
Nursing coverage	RN/LPN for medication administration (the Ohio rule)	Ongoing payroll
Staff training	First aid, personal-care, dementia & SMI training per staff	Per person
Background checks	BCI ~ \$30 + FBI ~ \$30 + vendor, per person	Per person
Insurance	GL + professional + abuse + property + auto + Ohio BWC workers' comp	Get quotes
ODH license	\$320 per 50 licensed persons (initial & annual)	Small home ≈ \$320
Business setup	SOS \$99 (no annual report), EIN (free), NPI (free), bank, legal	~\$99 + legal
ODA waiver certification	Application fee (~\$730 in 2025) + policies	Confirm current
Policies	P&P Manual + Forms Packet	One-time
Accreditation (optional but recommended)	CARF or Joint Commission (see 9.2)	Low-to-mid five figures over the cycle

9.2 If you pursue accreditation (recommended for brain injury)

- **CARF:** its Brain Injury specialty is layered on a base Medical Rehabilitation accreditation and requires about six months of operating data before survey. Commonly cited fees are a ~\$995 application fee plus about \$1,525 per surveyor per day, on a 1- or 3-year term — request a current quote. (888) 281-6531, carf.org.
- **The Joint Commission:** offers an Assisted Living Community accreditation (about a \$1,700 application deposit, then census-based fees on a 3-year cycle). Request a quote: (630) 792-5000, jointcommission.org.
- **It's staged:** both require you to be operating and serving residents before survey, then an initial on-site survey, then a decision. Plan roughly 12–18 months. The VA's brain-injury programs are CARF-accredited, which tells you what payers value.

9.3 Timeline

Early	Administrator & fire path	Confirm your administrator pathway; engage the State Fire Marshal on occupancy
Weeks	Business & home ready	Entity, insurance (incl. BWC), home to OAC 3701-16 specs, zoning
Weeks	Staff & policies ready	Nursing coverage, first aid, WebCheck, P&P adopted
60+ days	Apply → inspections → license	Application (≥60 days out) → fire + pre-licensure survey → RCF license
Then	Certify & enroll	ODA Assisted Living certification + Ohio Medicaid (PNM) enrollment
Ongoing	Referrals & full census	Area Agencies on Aging + hospital/rehab discharge planners

PART 10

Getting Your First Residents

In short: Waiver clients reach you through the Area Agencies on Aging, and through hospital and rehab discharge planners — OSU Dodd, MetroHealth, and the Level 1 trauma centers are the brain-injury pipeline. Build those relationships before you open.

10.1 The referral sources

- **Area Agencies on Aging (AAAs):** this is Ohio's most direct Medicaid referral channel — the AAA that certified you also assesses waiver clients and chooses placements. Once you're a certified provider, stay in close contact with each AAA's provider-relations and care-management teams; tell them your openings, service packages, and brain-injury capability.
- **Hospital & rehab discharge planners:** the biggest pipeline for brain injury — the trauma centers and neuro-rehab hospitals below discharge patients who need a residential home. Build the relationship with their case-management/social-work teams and ask how to get on their placement list.
- **Brain Injury Association of Ohio (BIAOH):** its HELpline and NeuroResource Facilitation program connect survivors and families to providers — a two-way relationship. HELpline (833) 783-1495; main (614) 481-7100; biaoh.org (professional-referral intake on the site).
- **Ohio 2-1-1 & the Medicaid hotline:** the statewide 2-1-1 line and the Ohio Medicaid Consumer Hotline (1-800-324-8680) route people needing services — get on their radar.

10.2 Trauma & neuro-rehab centers (verify before relying)

Metro	Program (referral source)	Contact (verify)
Columbus	OhioHealth Grant (Level 1 trauma); OSU Dodd Rehabilitation Hospital	Grant (614) 566-9000 / SW (614) 566-9265; Dodd (614) 293-4630
Cleveland	MetroHealth (Level 1) + Rehab Institute; University Hospitals (Level 1)	Metro case mgmt (216) 778-2075; UH (216) 844-1000
Cleveland	Cleveland Clinic Rehabilitation Hospital (brain-injury rehab)	(216) 455-6400 / (800) 223-2273
Cincinnati	UC Medical Center (Level 1); Daniel Drake Center (post-acute TBI)	UC (513) 584-1000; Drake via UC Health
Dayton	Miami Valley Hospital (Level 1); Rehabilitation Institute of Ohio	MVH (937) 208-8000; RIO (937) 424-8200
Akron	Summa Health (Level 1); Cleveland Clinic Akron General (Level 1)	Summa (330) 375-3000
Toledo	ProMedica Toledo Hospital (Level 1 trauma)	Main via promedica.org (verify)

Ask each referral source:

- How do you decide where to place brain-injury patients who need a residential home? What do you need from me to be on your placement list (license, certification, capability, availability)? Who is the discharge planner / social worker / care manager I should stay in touch with?

10.3 Your Area Agencies on Aging (major regions)

AAA	Region	Phone
Central Ohio AAA (COAAA)	Columbus / central	1-800-589-7277 • (614) 645-7250
Western Reserve AAA	Cleveland / northeast	1-800-626-7277 • (216) 621-0303
Council on Aging of SW Ohio	Cincinnati / southwest	1-800-252-0155 • (513) 721-1025
Area Office on Aging of NW Ohio	Toledo / northwest	1-800-472-7277
Direction Home Akron Canton	Akron / Canton	1-800-421-7277

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the administrator & the building

Who	Contact	What to verify / ask
ODH — Bureau of Survey & Certification	(614) 752-9524 · LICCERT@odh.ohio.gov	The RCF application packet; the current fee; the pre-licensure survey; the 60-day rule
State Fire Marshal / local fire dept.	com.ohio.gov/sfm · your local dept.	The occupancy classification; sprinkler/alarm requirements; locked-egress plan; the inspection
Ohio Board of Nursing / med-admin course	nursing.ohio.gov	The LPN medication-administration course, for your medication staffing
City/County Zoning	your jurisdiction	Allowed use at your address; group-home protection — in writing
Ohio Attorney General — WebCheck	ohioattorneygeneral.gov (WebCheck)	BCI/FBI background-check process and current fees
Ohio BWC (workers' comp)	bwc.ohio.gov	Setting up your state-fund workers'-comp policy

Business, insurance & billing IDs

Who	Contact	What to verify / ask
Ohio Secretary of State	1-877-767-3453 · sos.state.oh.us	Forming your LLC (Form 610, \$99); trade name (534A); no annual report
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPEs / CMS	nppes.cms.hhs.gov	Your Type 2 NPI and the right taxonomy
Insurance agent (Trusted Choice)	trustedchoice.com; carriers in Part 3.4	Coverages & limits; abuse & molestation; BWC setup
Business bank	Chase/Huntington/Fifth Third/KeyBank/PNC; a local credit union	Account fees, limits, business line of credit

Funding & referrals

Who	Contact	What to verify / ask
Ohio Dept. of Aging (ALW & certification)	1-866-243-5678 · aging.ohio.gov	ODA Assisted Living certification; the current per-diem; the room-and-board figure
Ohio Medicaid — provider enrollment (PNM)	ohpnm.omes.maximus.com · medicaid.ohio.gov	Enrolling your NPI; the Provider Certification Wizard; billing & timely filing
Ohio Medicaid Consumer Hotline	1-800-324-8680	Member eligibility & enrollment questions
Your Area Agency on Aging	find-your-AAA 1-866-243-5678 (Part 10.3)	Which AAA covers my area; the certification site review; how referrals & authorizations work
Accreditor — CARF / TJC	CARF (888) 281-6531 · TJC (630) 792-5000	Cost quote; the operating-first requirement; Brain Injury specialty

Advocacy, the state BI org & referral partners

Who	Contact	Role
Ohio State Long-Term Care Ombudsman	1-800-282-1206 · aging.ohio.gov/ombudsman	Independent resident-rights advocate — post the local number for residents
Adult Protective Services (county)	report via your county APS / 1-855-644-6277	Mandatory abuse/neglect/exploitation reporting
Brain Injury Association of Ohio	HELpline (833) 783-1495 · (614) 481-7100 · biaoh.org	NeuroResource Facilitation; survivor/family support; two-way referrals

OSU Ohio Valley Center for Brain Injury	wexnermedical.osu.edu	TBI research/education & a Columbus discharge relationship
Ohio 2-1-1	dial 211 · ohio211.org	Statewide information & referral — a no-cost inbound channel

You can do this

Opening an Ohio brain-injury residential care facility is a real build — a licensed RCF to OAC 3701-16 specs, a qualified administrator, trained staff and nursing coverage, and vulnerable residents — but it's a defined path, and the Assisted Living Waiver is built to fund exactly these residents. Take it one rung at a time: set up your administrator and your fire-code path, form the business, prepare the home and team, get licensed by ODH, get certified by the Ohio Dept. of Aging and enrolled with Ohio Medicaid, and earn referrals from the Area Agencies on Aging and the trauma/rehab centers. Verify each rate and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in ORC Chapter 3721, OAC Chapter 3701-16, and current ODA/Medicaid information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with ODH, the State Fire Marshal, the Ohio Dept. of Aging, your Area Agency on Aging, and qualified counsel before you rely on it. Confirm the current waiver per-diem and the room-and-board figure before you admit and bill.